TPF induction: docetaxel, cisplatin and fluorouracil

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

After induction · multidisciplinary team (MDT) selects definitive treatment

Active medicines:

Emetogenic risk: Minimal

Source-linked clinician/pharmacy draft. Confirm active phase, doses, routes, steroid totals and start/stop dates; no administration authority.

Additional phase-specific support

- After cycle 3, the multidisciplinary team confirms response, recovery and the definitive chemoradiation and/or surgery plan. No automatic radiotherapy date or universal subsequent regimen is authorised by selecting this induction route.
- Select the separately prescribed regimen when it is known; this review phase does not authorise further systemic anticancer therapy (SACT).

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00315 v7a · pNCCP 00315: treatment/supportive-care sections; NCIS v8 pp4–5 · NCCP 00315 with current NCIS and local supportive-care policy

Sources checked 2026-11-10 · SACTCheck v0.84.48 · Regimen-level drafts; prescriber and oncology-pharmacy verification required